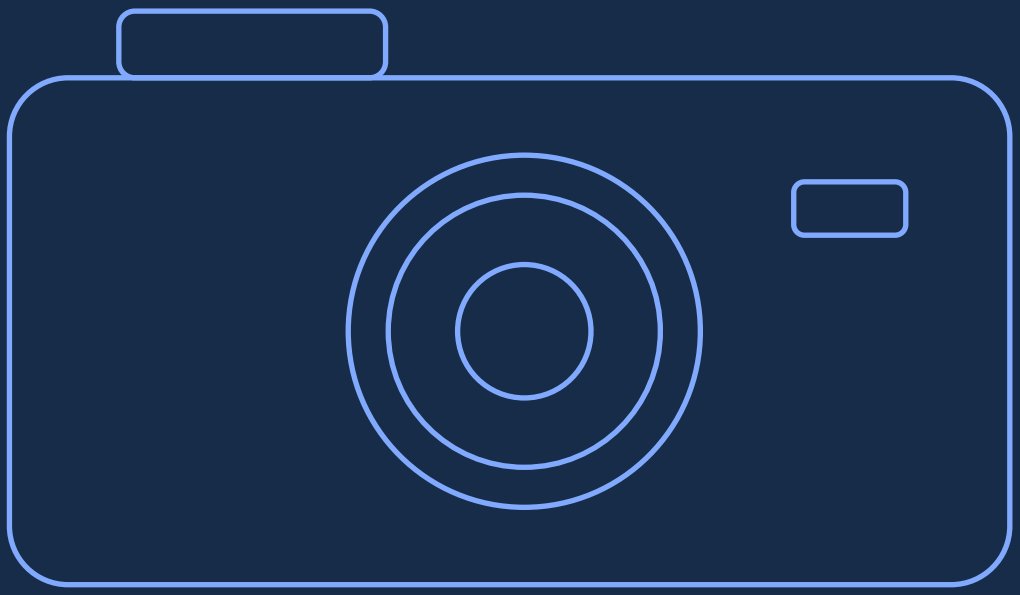
CLINICAL PHOTOGRAPHY

From first frame to portfolio.

A field guide for clearer images.
A workbook for a repeatable process.



06 MODULES / 12 LESSONS / 01 PORTFOLIO

Work with a dental model or another non-patient object. Diagrams are conceptual training illustrations, not patient photographs or clinical evidence.

Demonstration edition - October 2026. No approved education credits. Clinical delivery requires academy faculty review.

YOUR FINISH LINE

Make the process repeatable.

By the end, keep six things you can use again.

01 A baseline

One unplanned model frame, kept for comparison.

02 A setup card

Camera, lens, lighting, geometry and settings that worked.

03 A complete sequence

Frontal, right, left, upper and lower model views.

04 An improvement pair

The original and repeat, with one deliberate change.

05 A traceable handoff

Original files, a view index and transformation notes.

06 A next-session plan

The one variable you will test next.

Use this book as you learn

Read the lesson, try its task, then record what changed. Your portfolio is self-reviewed; the platform does not grade photographs.

FIELD GUIDE 01

The setup card

Write down the arrangement that worked, rather than guessing tomorrow.

Model / training ID and purpose

Camera or phone, lens and capture format

Working distance / magnification and orientation

Lighting type, position and background

Exposure settings and flash-sync check from the equipment manual

White-balance method and reference frame

What I will keep fixed in the next session

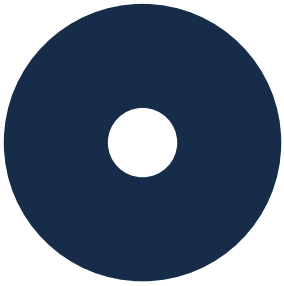
FIELD GUIDE 02

Sharp where it matters.

Aperture is one part of the focus-and-exposure decision.



Larger opening



Smaller opening

With other inputs unchanged, a higher f-number describes a smaller opening: less light enters, and depth of field generally increases. That does not guarantee a sharp frame. Close-up distance, motion, focus placement and equipment limits still matter.

A controlled comparison

Keep the model and camera in the same position. Make two frames with one setting changed. Compare the intended feature at the same viewing size; record which setting changed and what improved.

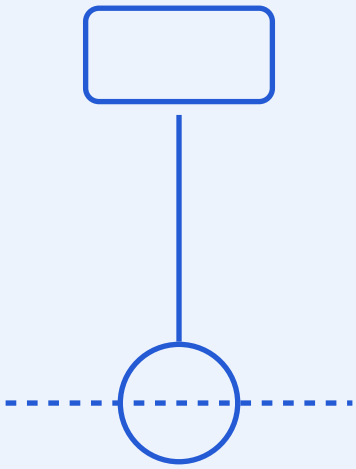
What changed? What remained unresolved?

Read more: Nikon, Understanding Maximum Aperture. Use your device manual for equipment-specific settings; there is no universal exposure prescription here.

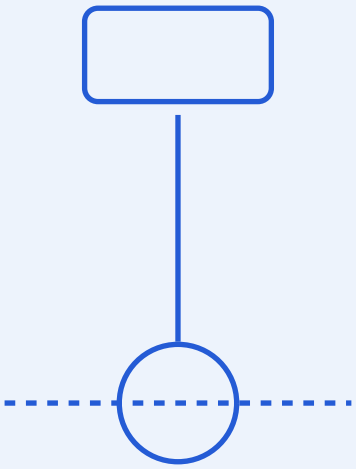
FIELD GUIDE 03

Geometry before comparison.

Two sharp images can still describe different camera-subject relationships.



A / baseline position



B / repeat this position

Match camera height, direction, distance and model orientation. A smaller crop after capture cannot prove that the original geometry was equivalent. Keep an honest note when conditions differ.

My repeatable position markers

Conceptual top-view diagram. Camera and model positions are illustrative, not clinical positioning instructions.

FIELD GUIDE 04

A five-view model sequence

Count intended views, not just the number of files.



01 / Frontal

Capture -> check orientation -> index the file



02 / Right lateral

Capture -> check orientation -> index the file



03 / Left lateral

Capture -> check orientation -> index the file



04 / Upper occlusal

Capture -> check orientation -> index the file



05 / Lower occlusal

Capture -> check orientation -> index the file

Use the subject-side convention in your capture record. Mirror reflections and transformed copies need explicit orientation notes. If orientation is uncertain, resolve it from the capture record or repeat the model view.

Training sequence only. Schematic markers are not examples of a clinically acceptable image.

FIELD GUIDE 05

The 60-second quality gate

Ask these questions before ending the capture session.

Purpose

Does this frame show the area or relationship I intended?

Clarity

Is the intended feature sharp enough at a useful viewing size?

Geometry

Are position, distance and framing repeatable?

Orientation

Can another person tell which view and side this is?

Coverage

Do I have each intended view, with no duplicates hiding a gap?

Provenance

Can I connect this image to its original and capture notes?

Repeat, rather than invent

If a reflection, obstruction or blur hides information, fix the capture and repeat. Editing cannot recover information that was never captured.

FIELD GUIDE 06

A handoff you can trust

Preserve the source and make the transformation visible.

ORIGINALS / unchanged

TRAIN-042_frontal_01.original - source capture, retained intact

DERIVED / if needed

TRAIN-042_frontal_01_crop.jpg - permitted crop, documented

INDEX / the connecting record

View | orientation | source filename | change made | review note

Use fictional training identifiers here. Real patient material belongs in your organisation's approved clinical systems under its local governance and permissions. Do not upload patient photographs into this course or workbook.

What should the next person be able to understand?

PRACTICE LAB 01

The repeatable workflow

Read the two lessons, then record your own observations below.

01 / Your first frame: a baseline worth improving

Photograph a simple object once without changing anything. Write down the purpose of the frame and three things you would need to reproduce it tomorrow. Keep this as your baseline, not a portfolio image.

02 / Permission and a clean working sequence

Write a five-step model-session checklist. Mark every point where a hand moves between the model area, camera, computer and phone. Redesign one avoidable crossover. Do not write a real patient name.

One useful decision

Which change will you keep? What evidence supports it? If you changed several things at once, isolate one variable in your next attempt.

PRACTICE LAB 02

Camera confidence

Read the two lessons, then record your own observations below.

03 / Build your kit, then write the setup card

Create setup card A in the workbook. Move the kit away, reconstruct it using only the card, and make a second frame. Note which missing instruction made reproduction difficult.

04 / Sharp where it matters: exposure and focus

Produce a pair of training frames labelled A and B. Write: what stayed fixed, what changed, what improved, and what became worse. Use the exposure experiment page in the workbook.

One useful decision

Which change will you keep? What evidence supports it? If you changed several things at once, isolate one variable in your next attempt.

PRACTICE LAB 03

Consistent light & framing

Read the two lessons, then record your own observations below.

05 / One light setup, believable colour

Make a four-frame experiment: baseline, changed light direction, changed white balance, then reconstructed baseline. Identify which difference is geometry and which is colour rendering. Keep the original files.

06 / Same subject, same geometry

Capture two model profiles: one in the planned position and one with a deliberately tilted model. Explain what could be misinterpreted if these were labelled “before” and “after.” Do not present the altered pair as treatment evidence.

One useful decision

Which change will you keep? What evidence supports it? If you changed several things at once, isolate one variable in your next attempt.

PRACTICE LAB 04

The five-view model sequence

Read the two lessons, then record your own observations below.

07 / Frontal and lateral views: build the sequence

Make a three-frame model set. Give each image a training-only case ID and view label. Ask a colleague to identify the views without seeing your capture sequence. Record any label or framing ambiguity.

08 / Mirror views: make orientation unmistakable

Create the two occlusal model frames. Intentionally make one common framing error on the model, explain why it fails, and then capture a corrected version. Include both in your local practice notes.

One useful decision

Which change will you keep? What evidence supports it? If you changed several things at once, isolate one variable in your next attempt.

PRACTICE LAB 05

Review, organise & hand over

Read the two lessons, then record your own observations below.

09 / The 60-second quality check

Review your five-view model set using the workbook rubric. Choose the weakest frame, record a single corrective action, repeat it, and explain whether the correction addressed the original failure.

10 / From memory card to a traceable image set

Create an index for your model set with filename, view, capture conditions, quality decision and any derived-copy changes. Write a three-sentence handoff that states what the set can and cannot demonstrate.

One useful decision

Which change will you keep? What evidence supports it? If you changed several things at once, isolate one variable in your next attempt.

Your portfolio challenge

Read the two lessons, then record your own observations below.

11 / Portfolio challenge: capture, critique, repeat

Use the capstone pages in the workbook. Self-check eight criteria: purpose, setup record, complete planned sequence, orientation, relevant sharpness, controlled exposure, traceable files and a justified correction. Ask an academy supervisor to review the portfolio before applying the workflow clinically.

12 / Your next session: make improvement repeatable

Write a 30-day practice plan: two model-based sessions, one peer review and one check of your local consent, equipment and storage protocols. Define how you will tell whether your process is more repeatable.

One useful decision

Which change will you keep? What evidence supports it? If you changed several things at once, isolate one variable in your next attempt.

PORTFOLIO CHALLENGE

Show the process, not just the result.

Keep your work locally. This rubric supports self-review, not faculty certification.

Your package: a setup card; five labeled model views; one baseline/repeat pair; a short view index; and a reflection naming one improvement and one limitation.

Repeatability

The setup and camera-subject relationship can be reproduced.

Circle one: Needs another attempt / Developing / Consistent

Coverage

All five intended model views are present and indexed.

Circle one: Needs another attempt / Developing / Consistent

Clarity

The intended features are visible, with no unresolved blur or obstruction.

Circle one: Needs another attempt / Developing / Consistent

Orientation

Side, view and any transformations are unambiguous.

Circle one: Needs another attempt / Developing / Consistent

Integrity

Originals remain intact and derived copies have a record.

Circle one: Needs another attempt / Developing / Consistent

Reflection

The chosen improvement is explained with a fair comparison.

Circle one: Needs another attempt / Developing / Consistent

YOUR NEXT SESSION

Turn learning into a routine.

Finish the assessment, then choose a next step you can actually repeat.

The one improvement I will test next

The setup details I must keep fixed

Further reading

Nikon: Understanding maximum aperture

Nikon: ISO control

Nikon: Setting white balance

GDC: How patient consent can help build trust and confidence (UK context)

GDC: Dental record keeping (UK context)

References support background reading; the exercises and conceptual illustrations are original. GDC material is UK professional context, not a substitute for local policy. No clinical care, diagnosis, treatment plan or accreditation is provided by this demonstration.